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THEORETICAL ARTICLE

Ethics and Regulation of Artificial Intelligence in Psychotherapy

Autonomy, responsibility, privacy, and the protection of vulnerable users under emerging European law

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ABSTRACT

Artificial intelligence (AI) is entering psychotherapy through conversational agents, clinical decision-support tools, and automated documentation. These applications raise ethical questions of particular urgency in a field built on trust, confidentiality, and personal encounter. This theoretical contribution examines five areas of concern: respect for patient autonomy and informed consent, the attribution of responsibility when algorithmic systems contribute to clinical decisions, the protection of highly sensitive mental health data together with transparency and the risk of systematic bias, the protection of vulnerable users in crisis situations, and the regulatory response. Drawing on principlism in biomedical ethics and on the broader literature on the ethics of algorithms, we argue that the ethical evaluation of AI in psychotherapy cannot be reduced to technical performance. We outline in general terms how the European framework, consisting of the General Data Protection Regulation, the Medical Device Regulation, and the Artificial Intelligence Act, addresses these concerns and where gaps remain. From a meaning-centered perspective, we emphasize that responsibility, a central category of Frankl's logotherapy, cannot be delegated to a machine. We conclude with principles for the responsible design, evaluation, and clinical use of AI in psychotherapy.

Keywords: artificial intelligence · psychotherapy ethics · responsibility · data protection · EU AI Act

1 Introduction

Psychotherapy has always relied on technologies of some kind, from the written case record to the telephone and video conference. The current wave of artificial intelligence (AI), however, differs in a decisive respect: systems based on machine learning and large language models no longer merely transmit or store communication, but generate responses, classify utterances, and produce predictions that may influence clinical decisions (Torous et al., 2021). Each of these functions touches on the core of the therapeutic relationship, which depends on trust and confidentiality.

Early overviews of the field pointed to questions of competence, liability, privacy, and the possible erosion of human contact (Fiske et al., 2019; Luxton, 2014). The present contribution takes up these questions and organizes them along five areas: autonomy, responsibility, privacy and transparency, the protection of vulnerable users, and regulation. Rather than offering a complete legal analysis, we aim to clarify the ethical structure of the problems and to indicate how European law responds to them. Throughout, we draw on the meaning-centered tradition, which places personal responsibility at the center of human existence (Frankl, 1946/1985).

2 Autonomy and Informed Consent

Respect for autonomy is one of the four principles that Beauchamp and Childress (2019) propose as a framework for biomedical ethics, alongside beneficence, nonmaleficence, and justice. In psychotherapy, autonomy is both the patient's right to decide on treatment on the basis of adequate information and a therapeutic goal, since many forms of psychotherapy aim to strengthen self-determination.

With regard to informed consent, patients must know whether they are communicating with a human being or with software, what the system can and cannot do, how their data are processed, and who is responsible for the service. Conversational systems are designed to produce fluent, apparently empathic language, which can lead users to attribute understanding and care where only statistical text generation is at work (Bender et al., 2021). Consent obtained under such misattributions is ethically fragile, even when formal requirements are met.

With regard to autonomy as a therapeutic goal, the concern is subtler. Systems optimized for engagement may encourage dependence rather than independence, and persuasive design features can steer users toward particular behaviors without their reflective endorsement (Floridi et al., 2018). An application should therefore be judged not only by short-term symptom reduction, but also by whether it supports the user's own judgment and, where indicated, encourages contact with human help.

3 Responsibility and the Responsibility Gap

When a therapist makes an error, established structures of professional, legal, and moral accountability apply. Their behavior results from training data, design decisions, and the context of use and is often not fully predictable even for developers. Matthias (2004) described the resulting situation as a responsibility gap: the more autonomously a learning system acts, the harder it becomes to attribute its outcomes to a particular person.

For psychotherapy, this gap is not merely a legal problem. Large language models can produce plausible but false statements, a phenomenon widely discussed as hallucination (Ji et al., 2023). If such a statement concerns medication, diagnosis, or a suicidal crisis, the consequences can be serious. A minimal ethical requirement is therefore that clinically relevant AI functions are embedded in a clear chain of human responsibility: the manufacturer for design and validation, the institution for the selection and monitoring of the tool, and the treating clinician for the decisions taken in the individual case.

From a logotherapeutic perspective, responsibility is not only an obligation imposed from outside but a constitutive feature of human existence. Frankl (1946/1985) understood being human as being responsible, that is, as the capacity to respond to the demands of a concrete situation. A machine can generate outputs, but it cannot answer for them in this existential sense. This supports the widely shared position that AI should assist, but not replace, the judgment of a responsible clinician, and it warns against clinicians deferring to algorithmic recommendations without examining them.

4 Privacy, Transparency, and Justice

Mental health data are among the most sensitive personal information. Under the General Data Protection Regulation, data concerning health belong to the special categories of personal data whose processing is prohibited unless specific conditions are met (Regulation (EU) 2016/679, 2016). Ethical concerns nevertheless extend beyond legal compliance: commercial providers may have incentives to use conversational data for product development, and the boundary between a health service and a consumer product is often unclear to users (Fiske et al., 2019).

Users should be informed when they interact with an AI system, and clinicians should understand the basis, limitations, and validation status of the tools they use. Complete explainability of complex models may be unattainable, but meaningful transparency about intended purpose, training data, known failure modes, and evidence is achievable and ethically required (Floridi et al., 2018; Mittelstadt et al., 2016).

Closely related is the question of justice. Algorithms trained on data from particular populations may perform worse for others, and seemingly neutral target variables can reproduce structural inequalities. Obermeyer et al. (2019) showed, for a widely used algorithm in health care management, that using health care costs as a proxy for health needs disadvantaged Black patients. Comparable risks exist in mental health care, where expressions of distress differ across cultural and linguistic groups. Systematic examination of performance across subgroups should therefore be part of every evaluation.

5 Vulnerable Users and Crisis Situations

People seeking psychological help, especially minors and people in acute crises, are often in a state of heightened vulnerability. A frequently cited study by Miner et al. (2016) found that widely available smartphone conversational agents responded inconsistently and often inadequately to statements about suicidal ideation, depression, and interpersonal violence. Although the systems examined were not designed as therapeutic tools, the study illustrates a general problem: users may turn to available technology in moments of crisis, regardless of its intended purpose.

Ethically, this means that providers of mental health applications bear particular duties of care. These include reliable detection of crisis signals, clear referral to human help and emergency services, protection against harmful content, and restraint in claims about effectiveness. People experiencing an existential vacuum, in Frankl's sense, may be especially receptive to a constantly available and affirming counterpart. A system that responds only with validation may relieve loneliness in the short term while leaving the underlying question of meaning untouched.

6 The European Regulatory Framework

In the European Union, three legal instruments are particularly relevant. First, the General Data Protection Regulation governs the processing of health data, including requirements for a legal basis, data minimization, purpose limitation, and the rights of data subjects (Regulation (EU) 2016/679, 2016). Second, the Medical Device Regulation applies to software whose intended purpose, as specified by the manufacturer, includes medical purposes such as the diagnosis, prevention, monitoring, or treatment of disease (Regulation (EU) 2017/745, 2017). Such software requires a conformity assessment according to its risk class before being placed on the market.

Third, the Artificial Intelligence Act, which entered into force in 2024 and applies in stages, introduces a risk-based framework for AI systems (Regulation (EU) 2024/1689, 2024). It prohibits certain practices, including AI systems that deploy manipulative techniques or exploit vulnerabilities related to age, disability, or social or economic situation in ways that cause significant harm. AI systems that are themselves medical devices or safety components of such devices, and that require third-party conformity assessment, are classified as high-risk and must meet requirements regarding risk management, data quality, documentation, human oversight, and accuracy. The regulation also establishes transparency obligations, for example that people must be informed when they interact directly with an AI system, unless this is obvious from the context.

Important gaps nevertheless remain. Many applications for well-being, coaching, or companionship are deliberately not marketed for medical purposes and thus fall outside medical device law, although they are used by people with mental health problems. General-purpose language models are likewise used for support they were not designed for. Regulatory classification therefore depends substantially on the declared intended purpose, whereas ethical evaluation must consider actual use. Professional ethics, clinical guidelines, and independent evaluation remain indispensable complements to law (Torous et al., 2021).

7 Conclusion

The ethical questions raised by AI in psychotherapy cannot be reduced to accuracy or efficiency. They concern the conditions under which persons can trust a helping relationship, make informed decisions, and remain the authors of their own lives. Several principles follow from the analysis: transparency about the artificial nature and limitations of a system; informed consent that takes the risk of anthropomorphic misattribution seriously; clear allocation of human responsibility; strict protection and minimal use of sensitive data; systematic examination of bias; robust crisis protocols; and independent evaluation before broad dissemination.

Regulation can enforce some of these principles, but it cannot replace the professional and personal responsibility of those who develop, recommend, and use such systems. The meaning-centered tradition reminds us that the decisive question is not only what technology can do, but what it is for. AI in psychotherapy is ethically justified to the extent that it serves the person, supports their capacity for responsible self-determination, and keeps open, rather than closes, the way to genuine human encounter.

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